

CIVIL ACTION NO, 07-CI-01303

**COMMONWEALTH OF KENTUCKY, ex §
rel. JACK CONWAY, ATTORNEY §
GENERAL, §**

PLAINTIFFS

v. §

PURDUE PHARMA L.P., et al., §

DEFENDANTS. §

**COMMONWEALTH OF
KENTUCKY**

PIKE CIRCUIT COURT

DIVISION II

DEPOSITION SUMMARY OF RICHARD S. SACKLER, M.D.

August 28, 2015

<u>Page Number</u>	<u>Testimony</u>
9-11	Introduction of parties and counsel for plaintiffs and defendants. Examination of deponent Richard Sackler, M.D. ("Dr. Sackler") by Tyler Thompson, Counsel for Plaintiff, State of Kentucky. Discussion of purpose of deposition and deposition instructions.
12	Discussion of Dr. Sackler's role at Purdue. Discussion of Defendants Purdue corporate entities, structure, and ownership. The Sackler family owns Purdue entities.
12-13	Discussion of Dr. Sackler's role that involves Purdue. Dr. Sackler has a role with Purdue Pharma. He also sits on the board of a consulting firm which consults to Mundipharma. The Sackler family owns Mundipharma.
14	Discussion of OxyContin and the companies involved in the production, manufacturing or distribution of OxyContin. Discussion of corporate structure, licensing of Purdue Pharma. Purdue does licensing agreements with other companies to sell OxyContin but do not own parts of those companies. Purdue owns companies that distribute or dispenses OxyContin. Further discussion of Dr. Sackler's role at Purdue.
15	Discussion of Declaration of Dr. Sackler dated July 30, 2014. The declaration states that Dr. Sackler is the general partner of Purdue Pharma, L.P. since 1990.
15-16	Discussion of Dr. Sackler's role in the production and marketing and promotion and training and management of Purdue sales representative for OxyContin. Dr. Sackler states that he was involved in the areas at a supervisory level, not an active level that began with the launch of OxyContin and ended in early 2003.
16	Further discussion of Dr. Sackler's declaration. Dr. Sackler states that it is accurately stated in his declaration that during the time period of 1996 to 2009, he was not directly involved with the day-to-day marketing or promoting of OxyContin, the training or

	management of Purdue sales representatives, or the scientific research into the conversion ration from MS Contin to OxyContin and those responsibilities fell to Purdue senior management in research and development, regulatory affairs, sales training and marketing, among others.
16-18	Discussion of an email dated May 31, 1999 involving Dr. Cornelia Hentzsch. Dr. Hentzsch was the general manager of Mundipharma Australia. Discussion of a drug called MS Contin. Dr. Sackler states it was controlled release and the majority of use (over 50 percent) was for cancer patients or malignant pain patients. Dr. Sackler states that the stigma that doctors prescribing morphine for anything other than cancer or malignant pain did not prevent doctors from prescribing it across the board. It was an inhibition to the use of a product in every application. Dr. Sackler believes that the stigma existed because of a popular understanding that morphine was an end-of-life drug, if it was to be used at all. Discussion of concerns of addiction and dependence of morphine.
18-20	Further discussion of MS Contin and OxyContin. Purdue companies had the exclusive right to sell MS Contin. Dr. Sackler states that Purdue Frederick and Purdue Pharma were involved in the early years of selling OxyContin. He also states there were no other Purdue companies involved in the selling of the product in the U.S. Initially Purdue Frederick has the exclusive right to sell MS Contin and then at some point Purdue Pharma acquired the rights to sell or if it did at all.
20	Discussion of the history of Purdue Frederick company structure. Dr. Sackler states that Purdue Frederick was the original company that his father and uncle acquired in 1952. It was a shell company and it was the first pharmaceutical company that they owned. Purdue Pharma was established in the early 1990s to take on new products and to also take on the risk of new products and also a few established products, but not all.
21	Discussion of the creation of OxyContin. Dr. Sackler states it was done in Purdue Frederick until the early 1990s when that responsibility was transferred to Purdue Pharma. He states there was a period of time in which once Purdue Pharma became involved that Purdue Frederick was involved, but Purdue Frederick was never involved nor Purdue Pharma in the manufacturing the drug, which was when it was developed, was manufactured by a company called P.F. Laboratories.
21-22	Further discussion of production of OxyContin. Dr. Sackler does not recall whether Purdue Frederick or Purdu Pharma both played a role in production or whether when Purdu Pharma took on the project it carried most of the weight or all of it.
22-24	Discussion of Purdue Frederick and Purdue Pharma organizational company structure.

	Dr. Sackler confirms there were differences between the Purdue Frederick and Purdue Pharma employees. He is not sure if there were differences in the Board of Directors. Dr. Sackler believes the training of the employees at Purdue Frederick and Purdue Pharma was the same.
24	Further discussion of developing MS Contin. MS Contin was developed in the late 70s and early 80s. Eventually they knew there would be competition for MS Contin.
25	Discussion of concerns developing OxyContin. Dr. Sackler recalls discussions of concerns related to how they were going to position OxyContin Controlled-Release for market share and whether they were going to position it and make MS Contin obsolete and take that market share that MS Contin had, or whether they were going to position it alongside MS Contin and sell them both together. He states that the principal goal was to produce the best product they could, and they believed when they started it and subsequently they believed it was and is a better product than MS Contin.
25-26	Discussion of a memo to Richard S. Sackler from Robert Kaiko, Ph.D. Dr. Sackler knows Dr. Kaiko. He states that Dr. Kaiko was the person who ran the project of developing OxyContin and was a clinical pharmacologist who was deeply involved in selecting formulations that would most likely to achieve the desired effect.
26-28	Discussion of document bates stamped PDD9520805292, Rational for Another Controlled-Release Opioid Analgesic. The document states “MS Contin may eventually face such serious generic competition that other controlled-release opioids must be considered. Other pharmaceutical firms are thought to be – to also be developing other controlled-release opioid analgesics.” Dr. Sackler states that it was a secondary or tertiary driver at that time.
29	Discussion of Exhibit 1, Declaration of Dr. Sackler. Dr. Sackler confirmed that the declaration states that he was not directly involved with the day-to-day marketing or promotion of OxyContin, the training or management of Purdue sales representatives, or the scientific research into the conversion ratio of MS Contin to OxyContin.
29-30	Discussion of Exhibit 2, an email from Dr. Sackler to Cornelia Hentzsch dated May 29, 1999. Dr. Sackler clarified that he is emphasizing “directly involved” and that he did not do any of the work or any of the training and he was not a salesperson. But as a senior executive, he was aware of what was going on and he consulted with other senior executives about what was going on and what should be going on.
30-31	Further discussion of the email. Dr. Sackler states in the email that he is committed to make OxyContin a huge success and he almost dedicated his life to it, and will have to catch up with his private life again. Dr. Sackler states the context of the email was to encourage and motivate Dr. Hentzsch, who was the head of the Australian business and was meeting with great success with MS

	Contin, to pay more attention than he thought she was paying to the prospects of potential for OxyContin.
31-32	Further discussion of the email, Exhibit 2. Dr. Sackler states that he was not aware that the company was committing a felony in how they were marketing and branding the drug. He said the physicians were extremely enthusiastic about the effectiveness and the safety and the reception their patients had, the response to the product. The goal was to make a better product than MS Contin, and this was one of the ratifications to that.
32-33	Discussion of Exhibit 3 which is the same as Exhibit 2, the email. Discussion of Purdue Frederick Company, Inc. pleading guilty to a felony charge of misbranding a drug, which was OxyContin, with the intent to defraud or mislead. Dr. Sackler states that he is aware and his understanding that the fraud occurred with respect to Purdue Frederick Company and not Purdu Pharma, L.L.P.
33-35	Discussion of an investigation rated to sales reps employed by Purdue Pharma making untrue claims. Dr. Sackler states that from the time they learned at top management levels that there was an abuse and diversion program, which was years before the settlement with the government, they launched multiple investigations both with inside resources and people and with external attorneys and others to identify if they had any sense mislead or caused this to happen. Enormous resources were spent to try to mitigate the problem whatever the cost was, in 2000 or 2001 and continued through the period of the plea with the U.S. government.
35-37	Discussion of Michael Friedman. Michael Friedman was the head of sales and marketing. Discussion of a memorandum from Dr. J.W. Watkins dated July 15, 1992 regarding a “Meeting with Shionogi held Wednesday July 24, 1992.” Dr. Sackler confirmed that at the time they were talking about doing business with Shionogi, a Japanese company, involving OxyContin Controlled-Release. Discussion of MST Contin Tablets and Oxycodone AcroContin Tablets. Dr. Sackler stated that it was their hope that OxyContin Controlled-Release would be well-received for pain – moderate to severe pain requiring opioids.
38-39	Discussion of Exhibit 4, bates-number PDDD9524706426, OxyContin launch team memo dated March 31, 1995. OxyContin was launched in January 1996. Dr. Sackler does not recall hearing about having any significant problem with MS Contin with respect to addiction, abuse, diversion or any of the problems experienced with OxyContin CR. Further discussion of the memo. The memo further states that Michael Friedman emphasized the threat that AB-rated generics posed to MS Contin. AB-rated generics will arrive and this is why it is of extreme timely importance that we must establish OxyContin. OxyContin can cure the vulnerability of the AB-rated

	generic threat, and that is why it is so crucial that we devote our fullest efforts to a successful launch of OxyContin.” Dr. Sackler states that it is not his belief that it was of extreme timely importance that OxyContin be established because AB generics were going to arrive and compete with MS Contin.
39-40	Discussion of Purdue Frederick or Purdue Pharma sales of OxyContin. Dr. Sackler does not know the sales profit. Discussion of the Forbes article “The OxyContin Clan. The 14 Billion Newcomer to Forbes 2015 List of the Richest U.S. Families.” Dr. Sackler states that two-thirds of Purdue Pharma sales is made up of OxyContin. Purdue Frederick does not sell anymore.
40-42	Dr. Sackler believes that a majority of the profits of Purdue Pharma come from OxyContin. Further discussion of OxyContin sales. Dr. Sackler believes the net sales are in the range of a billion dollars. Dr. Sackler states that there are no other companies in the United States that would be included in the one billion dollar sales. Dr. Sackler believes it is fair to say the Sackler family has made over a billion dollars off the sale of OxyContin.
43-45	Discussion of Exhibit 6, bates number PKY1738172006, a profit calculation for a Purdue entity. It states that the profit contribution in 2006 was 4 million – 4,718,767,000. Dr. Sackler states that profit contribution is not profit. Further discussion of the profit calculation breakdown.
46-47	Further discussion of Exhibit 4 regarding Oxycodone AcroContin, a controlled-release Oxycodone. Dr. Sackler believes that between 50 and 75 percent of patients could be adequately treated with MS Contin. This is based on his experience since 1980 and not any studies. Dr. Sackler is not aware of any studies on abuse liability for OxyContin before it was put on the market.
47-51	Discussion of Exhibit 7, Bates stamp PDD8801123847, a memo to Dr. Sackler from Paul Goldenheim, head of Research and Development regarding “The BfArM”, the German regulatory agency. Dr. Goldenheim responds to Dr. Sackler and states “Is this an opening to descheduling the agent?” It is Dr. Sackler’s understanding that descheduling means make it less restrictive. If it’s less restricted it would be easier for physicians to prescribe it. It would potentially increase sales since if it were Schedule III instead of Schedule II, physicians could prescribe by telephone. Dr. Goldenheim responds, “We do not have any abuse liability studies.” And this is as of 1997.
51-53	Discussion of abuse liability studies. Dr. Sackler states many abuse liability studies have been done to date, but they were done repeatedly for many formulations subsequently of both Oxycodone and of other abusable opioids, both in controlled-release form and in immediate-release form.

53-54	Dr. Sackler states they subsequently incorporated naloxone in some markets in Europe. They discovered it did not block the effect of the opioid but it did reduce the gastrointestinal side effects dramatically. By the time they had a full press to develop an abuse-resistant form of OxyContin they did do extensive work with another antagonist called naltrexone, and when it got released, block the effect of the opioid. But, unfortunately, after a huge investment, they could never be certain that it wouldn't be released when it was taken orally.
54-57	Discussion of Exhibit 9, Bates stamp PDD29520806439, an email from Dr. Sackler dated February 12, 1997. The email involved Walter Wimmer, general manager at Mundipharma-Germany. Mundipharma-Germany is owned by the Sackler family as well. Walter states to Bob in the email that due to discussions with BfArm he does see a 50 percent chance to get OxyContin off the narcotic drug status provided Bob could give some information on the very low abuse potential of our CR formulation.
57-59	Further discussion of email from Dr. Robert Kaiko dated February 27, 1997 where he was recommending against going uncontrolled but monitored with respect to OxyContin. Dr. Kaiko believes in the U.S. Oxycodone-containing products weren't as controlled at one time and they got abused, and that's the reason for now having controls. Dr. Sackler believes this is accurate. Discussion of opioid abuse, and morphine/heroin abuse. Dr. Sackler does not recall this memo or whether he read the whole email chain carefully or even saw it.
59-60	Further discussion of email. Dr. Kaiko states "Our dossier acknowledges a small handful of patients in our research program – who were suspect in terms of their drug accountability." Dr. Sackler does not know if it was reported that there were a handful of patients who were suspect in terms of their drug accountability but he is confident it was. He states if it was an FDA-submitted trial, it would have been.
60-62	Discussion of the Roth reprint. Dr. Sackler is not familiar. Dr. Sackler is unaware whether it was part of the plea agreement that Purdue Frederick had when they pled guilty to a felony. Dr. Sackler did not know they did not have had a post-marking abuse monitoring system or database and he doesn't believe it was a requirement by the FDA at the time. Dr. Sackler believes it would have been a good idea to have an abuse monitoring system and database to tell if it was being diverted or abused before putting OxyContin Controlled-Release on the market. Further discussion of Dr. Robert Kaiko's email regarding abuse of OxyContin and discussion of the experience with buprenorphine, which was initially uncontrolled which led to lots of bad press and controlled status.

	Dr. Kaiko was in charge of the development program of OxyContin.
62-66	Discussion of Dr. Richard Sackler's email response dated March 2, 1997. Dr. Sackler shares that Walter Wimmer had been very hesitant to pursue the development or marketing of OxyContin because he didn't believe it would sell very well. He turned out to be wrong and it did extremely well. Dr. Sackler was of the contrary opinion, but he said he came back and did quite a bit of work without any reference to anybody else on trying to get the BfArM to consider not scheduling it. Many in the U.S. were not enthusiastic about not scheduling it. In Germany there is no equivalent or a Schedule II. It is either an abusable drug and this have all the abusable drug controls or you were not. They were not in favor of this but felt that with the controls, which they thought would be appropriate and were appropriate, it would be very welcome, very useful to patients in the German market.
67	Dr. Sackler states that when he wrote back and said, "When we are next together we should talk about how this idea was raised and why it failed to be realized. I thought that it was a good idea if it could be done." but he didn't mean it. He just wanted to be encouraging and he was very glad it was closed.
68	Discussion of email dated March 12, 1997 regarding an inquiry to Paul, Michael related to C-II narcotic that is de-scheduled when naloxone is added. Dr. Sackler explains that he raised it just as a matter of information. They eventually did develop that product and it was extremely successful. At the time they researched it, they quickly discovered that naloxone didn't achieve the desired blocking effect, but they made another discovery that was even more valuable.
68-69	Discussion of Purdue Pharma's interrogatory responses. Dr. Sackler is unaware. He states he doesn't remember any studies conducted or anyone being retained to conduct studies regarding addiction and physical dependency rates of Oxycodone products prior to 2002. In 2002, Dr. Sackler was the president of Purdue Pharma, and this would not have required his approval or knowledge unless it led to something that was surprising or important and unexpected.
70	Discussion of Exhibit 9, memo dated December 14, 1993, Bates-stamp PDD9520509356. Discussion of marketing OxyContin Tablets. OxyContin Tablets will be marketed against Percocet and Duragesic. The OxyContin line may replace our MS Contin line if MSC generics are competing.

	Dr. Sackler states that MS Contin was used in treating both cancer patients and non-cancer patients and there was no focus, or consideration in this statement of whether it would be both.
71-73	Discussion of Percocet and Duragesic treatment. Further discussion of memo sections 2.3, 3.2, and 5.4. Discussion of quality of life questions in future studies. Dr. Sackler believes there were studies later that included quality of life measures but he is not certain.
73-74	Discussion of Oxycodone and morphine. Dr. Goldenheim testified that Oxycodone was twice as strong as morphine. Dr. Sackler believes Oxycodone is twice as potent as morphine. And they were very proud that they discovered this, in animal studies and in human studies, and they made it widely known perhaps even before the drug was introduced, and in the package insert and all the promotional material.
74-76	Discussion of Exhibit 10, memo dated August 10, 1992, Bates-stamp PDD9521410329 regarding Oxycodone Project Team Meeting Minutes. It states a literature search on oxycodone and oxymorphone is being conducted by one of the summer employees. Dr. Sackler believes it would have been a son or daughter of one of the people who worked for Purdue Frederick or Purdue Pharma who was doing the literature search initially. Dr. Sackler also states that it was a team decision to develop 20, 40, 80, and 160 milligram tablets in addition to the 10 milligram tablets that were already in the clinic at that time.
76	Further discussion of the memo regarding the package insert language. Dr. Sackler states the package insert is the Bible for the product. It is the core document from which all promotion or communication with physicians is to be based. It is a standard industry practice that the package insert is comprehensive and complete as possible.
77-85	Discussion of Exhibit 11, an email between Dr. Richard Sackler at Norwalk to Michael Friedman (head of marketing and sales) dated May 28, 1997. The email memo was to clarify their position on the very complex issues raised by Mike Cullen during the Phase IV team meeting and which were the subject of Dr. Richard's inquiry. Further discussion of the memo regarding oxycodone and morphine. It also discusses OxyContin, developing hydromorphone codeine, the non-malignant pain market. Dr. Sackler shares what he believes that non-malignant pain market is. He explains that the whole context and discussion of Mr. Friedman was not to stigmatize oxycodone in a way that morphine was stigmatized. Morphine was seen as an end-of-life extreme duress – paintent in extreme duress, often dying of cancer, but not only cancer. It was reserved by most physicians if it was used at all and it was often associated with a death sentence.

	They did not want oxycodone to change, the as Friedman says, personality of oxycodone, but you could say all the associated feelings of oxycodone, which were generally appropriate to a narcotic. They did not want that to be polluted by all of the bad associations that patients and healthcare givers had with morphine. Their actions in promoting the “twice as potent as morphine” never wavered. They never disguised it or hid it; they emphasized it.
85-86	Dr. Sackler testifies that they wanted to address both markets. Before they launched OxyContin, they thought that their sales would be about equally divided between cancer pain and non-malignant pain. They knew the market for non-malignant pain was much larger. Cancer is much less common than other pain states. They expected it would be about 50/50. Dr. Sackler learned it was about 20 percent of their sales, and thus he wrote Michael Friedman and said what’s going on here? Why aren’t we getting more cancer sales?
86-89	Discussion of Exhibit 12, an email from Dr. Sackler to Michael Cullen at Norwalk dated June 2, 1997. Michael Cullen advised Dr. Sackler that OxyContin is perceived by some physicians, particularly oncologists, as not being as strong as MS Contin. He further states that this perception has had some effect with physicians switching to MS Contin with more severe cancer pain patients, it has had a positive effect with physicians’ use in non-cancer pain. Since oxycodone is perceived as being a weaker opioid than morphine, it has resulted in OxyContin being used much earlier for non-cancer pain. Physicians are positioning this product where Percocet, hydrocodone and Tylenol with codeine have been traditionally used. Dr. Sackler states that it says weaker and stronger but they understood what it meant. Dr. Sackler was advised by senior employees that physicians perceived OxyContin Controlled-release as less strong than morphine. Dr. Sackler states these are words used but didn’t mean they believed it was less potent, because he knew they believed it was more potent. Their own practice recognized it was more potent. Percocet was 5 milligrams.
89-90	Further discussion of the email regarding the strong and weak association of OxyContin and morphine. Dr. Sackler believes that their sales force was telling physicians that it’s actually twice as strong as morphine and correcting that misperception that they had. Dr. Sackler state it was in the package insert, the promotion, in the conversion tables, and in the recommended dosing.
90	Further discussion of Michael Cullen’s email dated June 2, 1997, after the launch of OxyContin. Paragraph 3 says, “Since the non-cancer pain market is much greater than the cancer pain market, it is important that we allow this product to be positioned where it currently is in the

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